

SENATE BILL 2153

By Walley

AN ACT to amend Tennessee Code Annotated, Title 33
and Title 68, Chapter 1, relative to early childhood
mental health programs.

WHEREAS, the State of Tennessee recognizes the critical importance of early childhood mental health and family stability in fostering healthy, resilient communities; and

WHEREAS, young children exposed to adverse experiences, such as poverty, trauma, or parental mental health challenges, face increased risks of developmental delays, behavioral issues, and long-term health disparities; and

WHEREAS, evidence-based, home-based interventions that provide therapeutic support and care coordination can significantly improve mental health and developmental outcomes for young children while strengthening family resilience; and

WHEREAS, establishing a state program to deliver such interventions will expand access to critical services for Tennessee's most vulnerable families and complement existing initiatives; and

WHEREAS, the General Assembly seeks to partner with qualified providers, such as nonprofit mental health organizations, to implement this program efficiently and sustainably; now, therefore,

BE IT ENACTED BY THE GENERAL ASSEMBLY OF THE STATE OF TENNESSEE:

SECTION 1. Tennessee Code Annotated, Title 68, Chapter 1, is amended by adding the following as a new part:

68-1-2101. Early childhood mental health home visiting program established - Purpose.

(a) The department of health, in collaboration with the department of mental health and substance abuse services, is authorized to establish and administer an early childhood mental health home visiting program as a voluntary, evidence-based, and home-based intervention to promote the mental health, developmental progress, and family stability of children from birth to five (5) years of age and their families.

(b) The purpose of the program is to provide intensive, home-based services to families with young children facing significant stressors, including, but not limited to, poverty, trauma, domestic violence, or parental mental health or substance abuse challenges. The program must aim to:

- (1) Improve child mental health and developmental outcomes;
- (2) Reduce the incidence of child abuse and neglect;
- (3) Enhance parental capacity to support child development;
- (4) Strengthen family stability through coordinated access to community resources; and
- (5) Promote school readiness and long-term health equity.

68-1-2102. Program components.

An early childhood mental health home visiting program developed under this part must include, but is not limited to:

- (1) Home-based parent-child psychotherapy delivered by licensed clinicians trained in evidence-based, trauma-informed interventions;
- (2) Care coordination to connect families with medical, educational, housing, and social services;
- (3) Developmental and mental health assessments for children; and
- (4) Fidelity monitoring to ensure adherence to evidence-based standards established by the department of health.

68-1-2103. Program eligibility.

The department of health shall prioritize for eligibility families with children from birth to five (5) years of age who are at risk due to socioeconomic, environmental, or behavioral health challenges, as determined by the department of health in consultation with the department of mental health and substance abuse services.

68-1-2104. Program implementation.

(a) The department of health may contract with qualified nonprofit organizations, community mental health providers, or other entities to deliver program services.

(b) Providers shall employ or contract with licensed clinicians and care coordinators who are trained in the program's evidence-based model, as approved by the department of health.

(c) The department of health shall establish guidelines for provider qualifications, training, and program evaluation to ensure fidelity to the program's evidence-based standards.

68-1-2105. Funding.

(a) The department of health and the department of mental health and substance abuse services may seek and accept federal funds, state appropriations, grants, and private donations to support the early childhood mental health home visiting program.

(b) The program may be funded, in whole or in part, through TennCare, subject to agreements with managed care organizations for reimbursement of medically necessary services.

(c) To supplement funding of a program implemented pursuant to this part, the department of health shall explore partnerships with philanthropic organizations and other community stakeholders.

68-1-2106. Coordination with existing programs.

An early childhood mental health home visiting program implemented pursuant to this part must coordinate with existing state initiatives to maximize resources and avoid the duplication of services.

68-1-2107. Annual report.

(a) No later than January 15 following any calendar year during which an early childhood mental health home visiting program is implemented pursuant to this part, the department of health, in collaboration with the department of mental health and substance abuse services, shall submit a report to the general assembly detailing:

- (1) The number of families served by the program;
- (2) Program outcomes, including improvements in child mental health, family stability, and service coordination;
- (3) Funding sources and expenditures; and
- (4) Recommendations for program expansion or improvement.

(b) The annual report must be submitted to the chief clerk of the senate, the chief clerk of the house of representatives, and the legislative librarian. Publication and delivery of the report may be accomplished by electronic means.

68-1-2108. Rulemaking.

The department of health, in consultation with the department of mental health and substance abuse services, is authorized to promulgate rules to implement this part. Any such rules must be promulgated in accordance with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

SECTION 2. The headings in this act are for reference purposes only and do not constitute a part of the law enacted by this act. However, the Tennessee Code Commission is requested to include the headings in any compilation or publication containing this act.

SECTION 3. For the purpose of rule promulgation, this act takes effect upon becoming a law, the public welfare requiring it. For all other purposes, this act takes effect July 1, 2026, the public welfare requiring it.